



PROCEDURE

Clinical Escalation

Scope (Staff):	All staff
Scope (Area):	CAMHS-wide

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

Contents

Aim	1
Risk.....	2
Background.....	2
Definitions	2
Principles	2
Documentation Requirements for Risk Assessment.....	3
Escalation Criteria.....	3
Procedure	3
Post-Process Documentation Requirements.....	4
Appendix One: Escalation Flowchart.....	7

Aim

To ensure consistent and effective advice to all Infant Child and Adolescent Mental Health Services (ICAMHS) clinical teams and maintain safe patient care for complex, unstable and high-risk patients and/or clinical changes that impact on the ability to safely manage the risks.

This document should be read in conjunction with [CAMHS Risk Assessment and Management Policy](#).

Risk

Risk refers to 'the likelihood of an event occurring, which may have harmful outcomes for the person or others¹'. Failure to identify, assess and subsequently manage clinically significant risk may consequent in further adverse emotional, mental and physical health outcomes to self and others, and also a compromise of staff safety.

Having a clear escalation process for high risk situations, defined roles and responsibilities for the management increased clinical risk is paramount to the provision of consistent safe and transparent decision-making that is documented^{2,3}.

Background

In 2024 the Child Adolescent Mental Health Services underwent significant changes to the various functions structures. Moving into defined areas including ICAMHS North, East, South, Central and 5A Leadership.

Definitions

Child/ Young Person: An infant, child, adolescent, or young person under the age of 18 years in receipt of CAMHS services.

Key Stakeholders/ agencies: Professionals involved in the care of the child or support of the family, including but not limited to referrer, referring agency, school, group home, Department of Communities, Child Protection and Family Support, GP.

Risk Assessment: A gathering of information and an analysis of the potential outcomes of identified behaviours. It includes identifying specific factors of relevance to a consumer and their family or carers, and the context in which they may occur. This process requires linking historical information to current circumstance, to anticipate possible future change¹.

Risk Factor: Particular features of illness, behaviour or circumstances that alone or in combination lead to an increase risk of an adverse event or outcome occurring¹.

Significant Risk: High risk situations that can potentially impact the safe and consistent management of children and young people who are in our clinical care.

Structured Clinical Judgement: Judgement about risk based on a combination of the evidence base for risk factors, individual patient assessment including the child and family's view of their own experience and circumstances, together with clinical experience and knowledge of the consumer.

Principles

- The procedure clearly articulates the roles and responsibilities of health care professionals seeking support when escalating safety planning for complex, unstable or high-risk patient's.

- This procedure can be used when clinicians have a clear duty of care to intervene in the best interests of the child to support the safety of the child and others
 - When a mutual agreement to the safety plan cannot be reached with children and their families, their views are to be acknowledged, decisions discussed with them and continued efforts made to assist the individual regain control over their own safety. This may include recommendation of treatment in a more restrictive setting and/or consideration of treatment under the Mental Health Act or treatment at a more suitable time.

Documentation Requirements for Risk Assessment

When risk is identified following completion of the CAMHS Initial Assessment, a [CAMHS Risk Assessment and Management Plan](#) (CRAMP) must be completed on webPSOLIS. All sections must be completed.

Escalation Criteria

The application of this Escalation Procedure includes (but is not limited to):

- a. Any situation involving a complex, unstable, or high-risk patient.
- b. Patients with significant risk management concerns (Refer to: [CAMHS Risk Assessment and Management Plan](#)).
- c. Situations where clinical decisions exceed the comfort zone of the team's psychiatrist.
- d. Where there are disagreements between clinicians about patient care
- e. Where there is disagreement between ICAMHS and other health and non-health service

Procedure

Steps	Additional Information
1. Head of Department (HoD) inform the relevant ICAMHS Director of the situation if not already aware or vice versa	Document detailed notes on the patient's condition, decisions made and the rational in the patient's medical record ²
2. HoD and/or Director to escalate to the Director Clinical Services (DCS) for advice and support	Do not proceed to the next steps without discussing with the DCS. In the absence of the DCS, the Chief Executive (CE) will be the point of contact
3. HoD and/or Director to communicate the next steps / actions with the team leadership	Occurs when required to be raised in a timely manner. WI Escalating Clinical Issues- CAMHS Inpatient Service

Steps	Additional Information
4. HoD and/or Director to action agreed next steps as advised by DCS	All steps and actions to be clearly documented in the patient's medical record. Including any advice or discussions held with other agencies and service providers
5. All changes in the patient's condition or any new information to be provided to the DCS and documented in the patient's medical record.	Summary to be documented on a briefing note template and forward onto ICAMHS DCS.
6. DCS to notify and update the ED CAMHS as required	

Post-Process Documentation Requirements

All information is to be documented in the patient's medical record.

The closing update from the HoD and/or Director to the ICAMHS DCS is to be documented in the medical record and a summary using the [briefing note \(BN\)](#) document template clearly articulating an overview of the situation, actions taken, and the outcomes.

The post-process update is to be sent through to the ICAMHS DCS with the ICAMHS Executive Director copied into the communication. Noting if it is to be escalated to the Chief Executive, the information recorded will be the foundation for the BN submission.

The post-process information will be retained as per the [State Records Act 2000](#)⁴ and may be used as supporting evidence to demonstrate collaboration in the management and care of high risk young people in our care.

The briefing note will be allocated a unique identification number and filed in a secure access restricted location of the approved EDRMS by the ICAMHS Administrative services in Records Management under Mental Health – Operational Management - Briefing Notes.

References and related external legislation, policies, and guidelines (if required)
1. Chief Psychiatrist Office: CP Standards for Clinical - Care: Risk Assessment and Management (effective 30 November 2015).
2. DoH Statewide Standardised Clinical Documentation for Mental Health Services MP 0155/21 (effective 9 August 2021).

3. DoH Recognising and Responding to Acute Deterioration Policy (MP 017/22 – effective 22 January 2023)
4. WALW - State Records Act 2000

Related CAHS/CAMHS internal policies, procedures and guidelines
CAMHS Clinical Assessment Policy
CAMHS Consent Policy
CAMHS Risk Assessment and Management Policy
Increased Clinical Observation Levels in CAMHS Ward 5A Policy
Acute Inpatient Unit-Progressive Risk Monitoring Work Instruction
Escalating Clinical Issues - CAMHS Inpatient Service Work Instruction

Useful resources
Mental Health Act 2014
Health Services Act 2016 (section 26(2)(c))

This document can be made available in alternative formats on request.

File Path:	CD/25/19477		
Document Owner:	ICAMHS Director Clinical Services		
Reviewer / Team:	Director ICAMHS East, ICAMHS Leadership Group; ICAMHS Policy		
Date First Issued:	08/01/2025	Last Reviewed:	
Amendment Dates:		Next Review Date:	08/01/2028
Approved by:	ICAMHS Director Clinical Services	Date:	06/12/2024
Endorsed by:	Safety and Quality, Audit, Risk and Compliance Executive Committee	Date:	08/01/2025
Aboriginal Impact Statement and Declaration (ISD)		Date ISD approved:	
Standards Applicable:	NSQHS Standards:  Child Safe Principles: 1, 2, 3, 4, 5, 8, 9, 10		
Printed or personally saved electronic copies of this document are considered uncontrolled			
 Healthy kids, healthy communities Compassion Excellence Collaboration Accountability Equity Respect Neonatology Community Health Mental Health Perth Children's Hospital			

Appendix One: Escalation Flowchart

It is essential that the relevant Directors and the CAMHS Director of Clinical Services (DCS) are aware of any situations that may impact on the safe management of managing risk for complex, unstable or high-risk patients. The Head of Department (HoD) is to activate this procedure when any of the following are present:

- a. Any situation involving a complex, unstable, or high-risk patient
- b. Patients with significant risk management concerns
- c. Situations where clinical decisions exceed the comfort zone of the team's psychiatrist
- d. Where there are disagreements between clinicians about patient care
- e. Where there is disagreement between ICAMHS and other health and non-health services

Yes - present

No

Continue to monitor and assess

The HoD **immediately** informs the relevant ICAMS Director of the situation (if not already aware) or vice versa

The HoD and/or Director escalate the situation to the ICAMHS DCS for advice and support

Yes

Action plan and next steps formulated with HoD, relevant Director ICAMHS and the DCS

No - **STOP**

DCS to inform and update the Executive Director CAMHS – as required

HoD to:

- a. Clearly articulate and communicate agreed action plan to relevant staff
- b. Maintain open lines of communication throughout the process with the Leadership team(s)
- c. Implement agreed actions



Document:

- a. Agreed actions and all responses to the strategies in the patient's medical record – including any advice or discussions held with other agencies and providers
- b. Include detailed notes on the patient's condition, the decisions made, and the rationale behind them and outcome
- c. Post process, provide a summary overview of the situation, actions taken and outcomes to the DCS